

USRad Strategic Repositioning – Project Report

****August 3, 2026 | Internal Document | Not for Distribution****

Prepared for advisor review. Covers the self-funded employer repositioning, the contract architecture decisions arising from it, the public-facing artifact corrections required, and the HIPAA gap identified in the course of the work.

Companion documents: PSA Exhibit C (Business Associate Agreement, draft) and Implementation Plan v2 with Addenda A and B.

1. EXECUTIVE SUMMARY

USRad is shifting its primary commercial target from uninsured cash-pay patients to self-funded employers. The shift is additive rather than a pivot – the cash-pay lane is retained – but it surfaced a set of contract, marketing, and compliance issues that had been latent under the original single-payer-type assumption.

****Four decisions define the outcome:****

		Decision	
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	Funding standard Funding assurance precedes routing. Employer arrears invoicing is an exception requiring approval, not the default		
	Rate structure One contracted rate per center per modality across all funding lanes. Rates vary by market. USRad's fee moves from cost-plus-with-cap to flat per modality		
	Cash-pay posture Retained as a lane, retired as a company identity		
	Contract scope Broaden the existing Fully Funded Assignment definition rather than introducing new assignment types		

The governing thesis, adopted as a standing principle:

> ****USRad is a prefunded imaging network. Some assignments are prepaid by the patient. Some are prefunded by an employer. Either way, funding assurance is secured before the assignment is routed.****

****The most significant finding outside the repositioning itself:**** the Provider Service Agreement requires HIPAA compliance but never requires execution of a Business Associate Agreement. No BAA exists today.

Verified status: ****no PSA has been executed. One center began onboarding but did not execute. No PHI has been transmitted.**** This is therefore a pre-launch gap rather than an active compliance failure. It is also a hard prerequisite for network recruitment, since a center's compliance officer will not transmit PHI without an executed BAA.

The onboarding that began without a BAA available is worth noting as a near-miss. Nothing moved, but the gap is reachable through the existing flow rather than theoretical, and the remedy should be in place before onboarding resumes.

2. WHAT CHANGED, AND WHY

2.1 The market shift

USRad launched against the cash-pay segment: uninsured and underinsured patients purchasing imaging at transparent prices. That market is real and remains the base.

Self-funded employers offer materially better unit economics on the same infrastructure:

		Cash-pay		Self-funded employer	
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	Revenue pattern Transactional, per-patient Recurring, contracted, population-based				
	Deal size One study An employee population				
	Acquisition cost Per patient, ongoing Per employer, amortized				
	Channel Direct-to-consumer Benefits advisors and TPAs				

The competitive set is already there. Green Imaging, the clearest market reference, states that it serves over 1,500 employer health plans across roughly 5,000 facilities.¹

> ¹ As published by Green Imaging on its own website, August 2026. Not independently verified. Competitor-scale claims change; treat as directional.

2.2 The mechanic that matters

Employers commonly cover imaging at 100% with zero member out-of-pocket. This is deliberate plan design. The employer saves enough against the hospital-based allowed amount that waiving deductible and coinsurance still produces a large net saving, and the \$0 price is what actually steers the employee away from a hospital outpatient facility.

****This is the fact that broke the original model.**** USRad's platform, contracts, and marketing all assumed the patient prepays. Under employer sponsorship there is no member transaction at the point of service, so funding must come from the employer – either before routing or after.

That question did not exist under the original design. It was created by the shift.

2.3 The finding that narrowed the problem

Research into how the market leader actually operates changed the scope materially.

Green Imaging's flagship employer product engineers the claim out entirely: one monthly invoice to the employer, no claims to manage, a member voucher presented at the appointment. They position it as a bolt-on benefit that coexists with existing coverage rather than a plan-integrated carve-out.

The implication for USRad: ****for employers that accept the bolt-on prefunded structure, there is no new provider workflow.**** The imaging center uploads a report and is paid, exactly as in cash-pay. No 837, no repricing engine, no TPA integration, no accumulator or stop-loss plumbing.

This is conditional, and the condition is real. Employers that require imaging spend to credit member accumulators or count toward stop-loss will need plan integration, which reintroduces the claim. Larger employers with heavy claims experience care about this more than small ones, which is likely why the market leader supports both paths while leading with the simpler one.

For employers that accept the bolt-on structure, this collapsed roughly 80% of the platform build and contract rewrite the earlier analysis had assumed necessary. See assumption 8.3.

3. CONTRACT ARCHITECTURE

3.1 The question that was actually at stake

Prior PSA versions carried a pay-when-paid clause for claims-funded work:

> ****v6.1 § 4.4:**** USRad shall disburse Provider compensation within ten (10) business days following USRad's receipt of corresponding payor funds. USRad shall have no obligation to disburse amounts not received from the payor.

The initial recommendation was to replace this with an unconditional USRad payment obligation, on the reasoning that self-funded employers are not regulated insurers and a provider will not price that credit the way it priced a workers' compensation carrier.

****That recommendation was superseded by a correction from the founder.**** At AnciCare, across 40,000+ cases annually, the pay-when-paid clause never produced a dispute – because authorization preceded routing and authorization was reliable. The contract clause was a backstop on a risk already eliminated operationally.

****The control was the intake gate, not the contract language.****

3.2 The resolution

This reframes the question from **how do we allocate the risk** to **how do we prevent the event**.

Under a funding-assurance-before-routing standard, the employer has prefunded or is inside an active funding cycle before any assignment reaches a center. No credit is being extended. There is nothing to guarantee against.

The provider-facing statement is operational rather than financial, and stronger for it: ****USRad does not route an assignment until funding is secured.****

3.3 The narrowed PSA scope

Following advisor review, a broad Version 7.0 rewrite was rejected in favor of a minimal amendment.

****v6.1 § 1.7 already reads:**** *"Fully Funded Assignment" means an Assignment for which USRad has secured funding prior to the provision of services.** Nothing in that definition names the patient as payer. It already covers employer-prefunded work.

****Broaden, do not rename.**** Retaining the term avoids an ``assignment_type`` enum migration, portal badge
file:///home/usrad/Web Development/usrad-marketing/docs/workstream-a/source/2026-08-03-project-report.md

changes, and a migration addendum for any provider already executed under v6.1.

A proposed routing-gate warranty was also ****deleted rather than reworded****, on the advisor's observation that the drafted phrasing implied USRad owed only where the gate failed. On review, the warranty was unnecessary: v6.1 § 4.3 already states an unconditional payment obligation on Fulfillment Complete. The funding gate remains internal routing policy. Placing it in the PSA would create a provider breach claim for something causing the provider no harm.

****Resulting scope:****

Action	Item
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Broaden	§ 1.7 – clarify funding may be secured from member, employer, sponsor, or other approved source
Preserve	§ 4.3 payment trigger, unchanged
Deactivate	Dormant claims-funded requirements
Fix	The v6.1 defect list (§ 3.4)
Add	Anti-circumvention; Exhibit C (BAA)

****No Article IV structural change. No new assignment types.****

3.4 Pre-existing v6.1 defects

Present in the current agreement, independent of the repositioning.

Defect	Location
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§ 4.1 states rates apply "regardless of Assignment type"; Exhibit A states they apply "exclusively to Fully Funded Assignments." Both cannot be true	Article IV / Exhibit A
§ 4.5 Member Non-Collection absent from survivability list	§ 9.1
§ 2.7 requires electronic claim submission for work that generates no claim	§ 2.7
No BAA requirement anywhere in the agreement	Article II
No anti-circumvention provision	Article VI or new
Exhibit A "Program Expansion" clause overrides Article VIII negative consent	Exhibit A

Anti-circumvention becomes material the moment USRad introduces a center to an employer.

4. THE HIPAA GAP

4.1 The finding

PSA § 2.5 requires the provider to maintain PHI confidentiality in accordance with HIPAA. The agreement never requires the parties to execute a Business Associate Agreement, and none exists.

USRad receives patient identity, physician orders, exam types, and signed radiology reports, and performs assignment coordination, report routing, and payment administration on behalf of contracted centers. That is business associate activity.

****Verified status:**** no PSA executed; one center began onboarding without completing execution; no PHI transmitted. This is a pre-launch gap, not an active compliance failure.

The distinction matters for sequencing. Because nothing has moved, USRad can put the BAA, the vendor chain, and the underlying security program in place before the first disclosure rather than remediating afterward – an advantage that disappears the moment onboarding resumes.

4.2 Why it is a recruitment blocker

A center with a functioning HIPAA program will not transmit PHI without an executed BAA, and will ask for one during onboarding. Absent a document to present, onboarding stalls at the point where the commercial argument has already been won.

4.3 The remedy

A BAA has been drafted as ****Exhibit C to the PSA**** (attached separately). Structural decisions:

- ****Exhibit, not body text.**** BAAs amend on a different cycle than commercial terms, and a center's compliance officer often reviews the BAA separately from the business agreement.
- ****One signature.**** Exhibits form part of the agreement they attach to; PSA § 9.3 already states this. The provider signs once, in the same DocuSeal envelope.
- ****Two conforming PSA changes required:**** § 2.5 must reference Exhibit C; § 9.1 must add it to survivability.

Four provisions require decisions before execution, flagged in the draft:

file:///home/usrad/Web Development/usrad-marketing/docs/workstream-a/source/2026-08-03-project-report.md

Provision	Decision
§ 2.4 De-identification	Load-bearing for MarketScope, employer utilization reporting, and network benchmarking. Some compliance officers push back; a common compromise limits disclosure to aggregate form not naming the center
§ 4.2 Breach notification	Ten business days drafted. Five is more provider-friendly and may be requested
§ 5.2 Subcontractor list	Standard and increasingly requested. Requires the downstream vendor chain to actually be papered
§ 9.4(c) Retention	Must match what the platform's data lifecycle actually implements

4.4 The obligations behind the document

The drafting is inexpensive. Performance is not. Execution commits USRad to direct Security Rule liability:

- Documented risk analysis
- Written security policies and procedures
- Workforce training and sanctions policy
- Designated security official
- Incident response capable of meeting the notification deadline
- Executed BAAs with every downstream vendor touching PHI
- Access, amendment, and accounting request handling within stated windows

****Recommended approach:**** one-time consultant engagement for the initial risk analysis and policy set; a compliance platform for training records and vendor tracking; everything ongoing in-house. Full outsourcing is not justified at current scale.

USRad's PHI footprint is narrow – assignment data, orders, reports, payment records. Not an EHR, not a clearinghouse, not storing images at scale. That keeps the risk analysis scope genuinely small.

4.5 The vendor BAA inventory – most likely gap at launch

Infrastructure is the obvious layer: Supabase (database, ePHI at rest) and Vercel (PHI in transit, request logs) both require BAAs, and both likely gate them behind a paid tier.

Authorize.Net is a useful boundary case. Payment processors generally argue they handle financial data under PCI DSS rather than PHI. That holds only if no clinical detail is passed in transaction metadata – an internal assignment ID rather than a procedure description keeps them out of scope.

The commonly missed categories are not infrastructure:

Category	Exposure
Transactional email and SMS	PHI in message bodies and retained logs
Error monitoring	Captures request payloads on exception
Analytics and session replay	Records form fields as typed
Support tooling	Patients paste clinical detail into chats
AI/LLM vendors	Most consumer tiers explicitly disallow PHI
Backups	Follow the data

****Two architectural moves available because nothing is live:**** scrub PHI before it reaches optional vendors (strip request bodies from error monitoring, disable session replay on authenticated routes), and design notifications that carry no PHI ("details in your portal" rather than naming the study). Every vendor kept out of scope is one fewer BAA to obtain and audit.

****Employer side, not covered by Exhibit C:**** a self-funded group health plan is itself a covered entity. Employer arrangements involving eligibility files or utilization data require their own BAA, belonging to the employer agreement document set.

5. PUBLIC-FACING ARTIFACT AUDIT

Full audits of the provider and employer pages, and both downloadable PDFs, were conducted. Findings are detailed in Implementation Plan v2 and its addenda. Summary of what matters at advisor level:

5.1 The two pages contradicted each other

The provider page promised "no claim filing" in four places. The employer page told employers "imaging centers bill the program directly." Both were live. A center's CFO reading both before signing would find the conflict.

Notably, the employer page stated ****no funding timing at all**** across thirteen components – no deposit, file:///home/usrad/Web Development/usrad-marketing/docs/workstream-a/source/2026-08-03-project-report.md

invoice, net terms, or funding cadence. The exposure was one-directional: the provider page over-promised into a gap the employer page left empty. That gap is where the prefunding standard gets stated, at no cost, because nothing has to be retracted.

5.2 Unit economics did not reconcile

The provider brief claimed ****\$375 net margin per scan****. The employer ROI report showed ****\$350 per scan**** paid to USRad. As written, the spread was negative.

Root cause: both documents applied an MRI-tier figure and labeled it a modality-mix average. The ROI report's own scan-level table showed MRI at \$260-350, CT at \$250-350, ultrasound at \$150-250, X-ray at \$75-150. A genuine blended average lands nowhere near either headline number.

****Reconciled:****

Value
Center per-scan reimbursement, plain MRI
USRad fee, flat per modality
Published global price
Volume basis
Realistic monthly add to center

Two consequential corrections follow. The provider page's headline revenue figure drops from ****\$37,500/month to \$3,400-\$10,000**** – a 75-90% reduction, following from standardizing the volume basis and correcting the per-scan figure. And "net margin" is replaced with "per-scan reimbursement," since USRad cannot know a center's margin.

The incremental-margin argument the \$375 figure was reaching for is retained, in prose where it cannot be arithmetically falsified: **fixed costs are already covered; volume into an unfilled slot carries only the read fee and supplies.**

5.3 Compliance claims requiring removal

Present on the employer page and in the generated ROI report, addressed to executives whose counsel will verify them:

Claim	Issue
"Licensed in all 50 states"	No license type named; unsupportable
"HIPAA certified – SOC 2 Type II compliance"	HIPAA certification does not exist. No SOC 2 report exists
"ERISA compliant – structured as permissible benefit"	Legal conclusion asserted flatly
"Workers' comp approved"	Regulatory approval claim for a deferred product
"Average client saves \$1.7M in year one"	Asserts live employer clients; none exist
"Implemented this exact model 1,200+ times"	Center-recruitment count reused as implementation count

These were not caused by the repositioning and are not fixed by it. They have been promoted into the first release.

5.4 Historical figures

Several AnciCare figures are extrapolations presented at precision they cannot support, and two conflict across artifacts (\$180M vs \$160M for the same fact).

Recommended treatment is presentation rather than disclaimer – round down and state as approximate: "over \$150 million" in provider volume, "150,000+" patients served, and "AnciCare 1994-2002" as the company fact rather than a career-span figure. The 1,200+ center count is documented and needs no hedging.

5.5 Scope discipline

The audits identified six components requiring structural rebuilds – diagram node counts, grid geometry, calculator restructuring. ****All are driven by the claims model.**** Under the prefunding standard, none is necessary. They have been explicitly parked.

6. IMPLEMENTATION STRUCTURE

Three releases, per advisor recommendation.

Release	Purpose	Contents
1	Correct what is false	Prepaid vocabulary; four claims-workflow statements; compliance claims; WC availability claims; both PDFs

| ****2**** | State the employer proposition | Employer as buyer; \$0 as steering mechanism; funding language; rate labeling |
| ****3**** | Align contract and platform | PSA amendment; Exhibit C; v6.1 defects; minimum funding gate specification |

****Governing constraint:**** change only what is false, contradictory, or blocking.

****Sequencing note:**** the employer ROI report is generated code, not a static PDF. Its Release 1 and Release 2 changes should be executed in a single pass, since the compliance corrections and the rate language describe the same commercial terms.

7. OPEN ITEMS

7.1 Requiring decision

Item	Owner
Flat fee table for remaining modalities (plain MRI settled at \$75)	Founder
Gross vs. net revenue treatment	Accountant
Partial member cost share – supported or 0%/100% only	Founder
Exhibit C decisions: de-identification scope, breach timing, subcontractor list, retention	Founder + counsel

7.2 Requiring counsel

Item	Note
Exhibit C review	Legally required document with direct statutory liability
Fla. Stat. § 626.88 administrator licensure	Whether the intended employer money-movement structure requires a certificate of authority. Materially lower exposure under prefunding with no member collection and no plan funds held than under a claims model
Compliance claim replacements	Particularly the plan-structure and data-security language

7.3 Track 4 – Employer Contracting Architecture

****The employer agreement does not exist.**** All three releases leave USRad with a truthful site, a clean PSA, and an executable BAA – and nothing to sign an employer with.

Established as a ****parallel track rather than a fourth release****, so it does not remain an indefinitely acknowledged gap while marketing and provider work proceed. Document set:

- Employer Services Agreement
- Funding Arrangement
- Employer Rate Exhibit
- Employer-side BAA (Plan as Covered Entity)

Work begins with an outline rather than full legal drafting, pending counsel's resolution of the money-flow structure and the § 626.88 question.

This is correct sequencing, since provider recruitment is the near-term goal and does not require it. But it is the item standing between USRad and revenue.

8. ASSUMPTIONS THAT MAY BE WRONG

Listed for challenge.

****8.1**** That employers will accept prefunding. No employer has been asked. If the market insists on arrears invoicing, the funding standard and the provider promise both need revisiting.

****8.2**** That providers will accept the terms. No provider has been recruited. The payment structure may not be the binding constraint on recruitment – rate may be.

****8.3**** That the bolt-on structure is available. Some employers will require plan integration for accumulator and stop-loss credit. That reintroduces the claim and with it the platform build. ****This is the assumption on which the largest single scope reduction in this package rests.**** It should be tested in the first employer or advisor conversation, with one question: does imaging need to run through the plan document, or can it be purchased alongside it?

****8.4**** That the corrected revenue figures still recruit. Dropping the provider page headline from \$37,500 to \$3,400-\$10,000 monthly is a large reduction in the stated opportunity. It is defensible and beatable, but it is untested as a recruitment message.

****8.5**** That the self-funded employer market is the right target at current network scale. Employers ask about coverage density early, and the network is small.

Summary judgment

The repositioning is sound and the work required is smaller than initially assessed, principally because the market leader's own structure suggests that for employers accepting a bolt-on prefunded arrangement, the claim can be engineered out rather than built for. Where an employer requires plan integration, the claim returns – and so does the build.

Two items are genuinely urgent and neither is caused by the repositioning: the unsupported compliance claims currently published to executives, and the absent BAA that blocks provider recruitment.

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